جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : امل محمد عبد السلام احمد
المحافظة : القاهره **الحي / المدينة :** حلوان **العنوان :** عرب غنيم
رقم البطاقة : 28511180102922 **رقم التليفون :** 01228092487 **تاريخ تسجيل الإستمارة :** 20-10-2026
العمر(تاريخ الميلاد) : 18-11-1985 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي العلاج المساعد لوقاية العظام للمرضي الذين يتلقوا علاج هرموني حمض الزيلودرونيك [14157] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: امل محمد عبد السلام احمد
National ID: 28511180102922

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Zoledronic acid for bone protection during hormonal therapy**

التاريخ : **05-07-2026**	**الوقت :** -- --:--
	الطبيب المعالج : د/شيماء لاشين





National ID - Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

جوستاف روسي

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

البنك المصرية هيئة صحة مصر

01/28/2026

MDT FORM

Name:	Amal Mohamoud Abd Elsalam Mohamoud	**MRN:**	5594
Age:	40 Y	**ID:**	3931/2025
Menopausal Sta	Premenopausal	**Phone:**	01100493062
NRS SCORE:		**Marital Status:**	MARRIED
BMI:		**Family History:**	
CO Morbidity:	MF	**Reason Of MDT:**	New case (metastic)

Clinical Data:

BX: IDC & ILC G2

ER: 8/8 **PR:** 8/8 **HER2:** +2 not ar **KI67:** 70%

Sonomammography:

ACR D
Rt post. 1/3 ill defined mass US>> 4*2.8 cm at 11-1 o'clock with suggestive ms invasion
marked edema and skin thickness
Level 1-2 pathological ln largest 2.2 * 1.2 cm at level 1

O/E>> T4b N2 M1
With palpated swelling laterally just bellow the cancerous breast
د/ دينا عبدالرحمن

Mets Workup:

CT CAP with contrast >> Rt hepatic lobe seg 5met lesion
multiple lytic and sclerotic osseous lesion (metastatic)

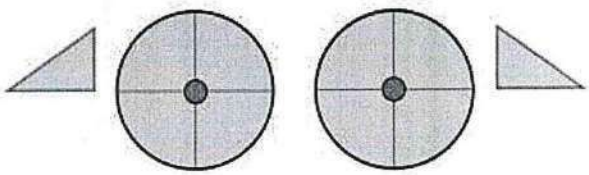
History:

Recommendations:

MDT Outcome & Plan

DeNovo Luminal Her2-ve MBC to bone & liver, for:
1.Bone scan
2.Meanwhile, start 1st line Zoladex/AI (Femara) + Zometa q3months

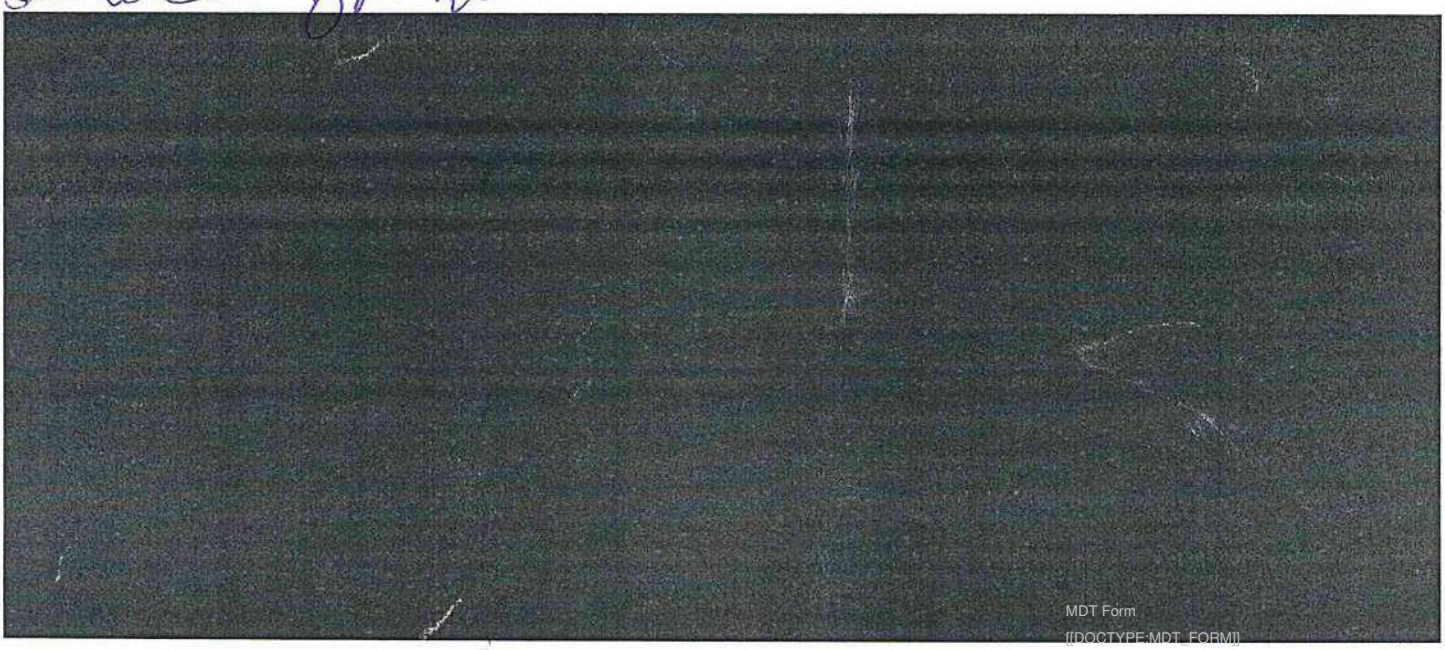
R L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
Shaimaa Lasheen	Omneya Essawy	Mona Zaki			



Name: امل محمد عبد السلام احمد	
MR. : 5594	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient has a diagnosis of primary right breast cancer, clinical stage T4b N2 M1 with bone metastases. Imaging shows a small right hilar lesion measuring 6 mm. Pathology reports estrogen receptor (ER) 8/8, progesterone receptor (PR) 8/8, and HER2 SISH not amplified. The patient currently has no approval for blood transfusion.

The patient has pending bone scan results. There is right breast induration for which the plan includes Zoladex, Zometa, and Arimidex with close follow-up.

The patient has metastatic breast cancer to bone and is receiving Zoladex, supportive care, and Arimidex for palliation. A bone scintigraphy performed on 14/5/2026 demonstrated features of widespread metastatic disease.

The patient is chronic on tramadol, paracetamol, and methyl-techno; the regimen appears well tolerated and will be continued unchanged.

The patient attended today for Zometa labs. She received palliative radiotherapy to both hip joints at El-Hussein Hospital, total dose 20 Gy in 5 fractions with a 1-week interval.

تحريرا في ,

التاريخ :	21-09-2026	**الوقت :**	

الطبيب المعالج : dina makram

يعتمد ,

GUSTAVE ROUSSY INTERNATIONAL EGYPTE

Patient Name	: الأستاذه / امل محمد عبد السلام احمد	Request No.	: 13003907
Patient No.	: 28511180102922	Request Date	: 10/12/2025
Path No.	: 3827/25	Verification Date	: 20/12/2025
Sex	: Female	Financial Category	: جوستاف روسي - مبادره
Age	: 40 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right breast mass.
Nature of Specimen: Tru-cut biopsy, collected on 10/12/2025, at 10:50 am.

Gross Examination:
- Four fragmented grey white tissue cores, the longest is 1.5 cm. Totally submitted.
Technique:
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for; ER clone (1E2), PR clone (SP1), Her2 clone (4B5), Ki67 clone (30-9) & SISH.

Microscopic Examination:
* Histological type: Mixed invasive duct & invasive lobular carcinoma.
* Grade 2 (differentiation 3, nuclear pleomorphism 2, mitosis 2).
* DCIS: Absent.
* LCIS: Absent.
* Angioinvasion: Absent.
* Perineural invasion: Absent.
* IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 80% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 70% of tumor cells (score 8/8).
- Her2: Intermediate incomplete membrane reaction in more than 10% of tumor cells (score 2+).
- SISH: Not amplified.
(Average Her2 copy No.: (1.7), Her2/CEP17 ratio: (1.13)).
- Ki67%: 70%.

Diagnosis:
Right breast mass, Tru-cut biopsy:
- Mixed invasive duct & invasive lobular carcinoma, Grade 2.

Verified by :
Dr.Duaa Mohamed

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY INTERNATIONAL EGYPTE

Patient Name	: الأستاذه / امل محمد عبد السلام احمد	Request No.	: 13003907
Patient No.	: 28511180102922	Request Date	: 10/12/2025
Path No.	: 3827/25	Verification Date	: 20/12/2025
Sex	: Female	Financial Category	: جوستاف روسي - مبادره
Age	: 40 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

IPT:
- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Equivocal (score 2+).
- SISH: Not amplified, Group V.
(Average Her2 copy No.: (1.7), Her2/CEP17 ratio: (1.13)).
- Ki67%: 70%.

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%
and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.
IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of
the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.
ISH groups:
The ratio is calculated and the results is interpreted as:
• Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 4.

Verified by :
Dr.Duaa Mohamed

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY INTERNATIONAL EGYPTE

Patient Name	: الأستاذه / امل محمد عبد السلام احمد	Request No.	: 13003907
Patient No.	: 28511180102922	Request Date	: 10/12/2025
Path No.	: 3827/25	Verification Date	: 20/12/2025
Sex	: Female	Financial Category	: جوستاف روسي - مبادره
Age	: 40 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

• Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is less than 4. (Recounting give the same result).
• Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 6 (Recounting give the same result).
• Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 4 but less than 6. (Recounting give the same result).
• Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than 4.

Signature
Dr. Walaa Ibrahem Ghoneim.
Dr. Heba El-Shafeey.

Verified by :
Dr.Duaa Mohamed

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient ID: 90497	Patient Name English: Amal Mohamed Abdalsalam Ahmed
Age: ٤٠ سنة	2025/12/08 11:58:17 AM :Report Date

Women's Health Outreach Program

Bilateral Full field Digital Mammography and high-resolution ultrasound.

Clinical summary: 40 years old female patient with right breast lump.

Detailed Findings

Right breast:

- Dense fibro glandular densities (ACR density d).
- The right breast shows posterior 1/3 large ill-defined dense mass lesion with obscured margins and architectural distortion
- Marked parenchymal oedema and thickened skin identified
- **Ultrasound** revealed large ill defined hypoechoic mass lesion measures about 4.0 x 2.8 cm at its maximum diameters and located at 11-1 o ° clock position. It is seen focal no line of cleavage with the underlying muscle ??suggestive of muscle invasion. Associated overlying skin thickening measures about 4 mm.
- Normal nipple/areolar complex.
- Multiple enlarged axillary lymph nodes at level I, II with effaced hila, largest one measures about 22 x 12.8 mm at level I.

Left breast:

- Scattered fibro glandular densities (ACR density B).
- No masses, asymmetric densities, distortion or suspicious clusters of micro calcifications.
- **Ultrasound** demonstrated prominent glandular elements of the breast tissue with mottled appearance.
- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Non-specific axillary LNs.

Impression:

- **Right breast (BIRADS 5): Right UOQ malignant mass lesion with ipsilateral pathological axillary lymph nodes as described. For tru cut biopsy and dynamic MRI breast study.**
- **Left breast (BIRADS 2): fibroadenosis .**

Much obliged,
Dr. marwa a . el seoud

Mammogram
[[DOCTYPE:MAMMOGRAM]]

NILE SCAN & LABS

مركز النيل للأشعة و التحاليل

Name : Amal Mohamed Abdel Salam Ahmed
Branch : Dokki
Ref.Physician Prof : Goustav Roussey Egypt

Date : 14-05-2026 09:53
Medical No. : 374803

Whole-Body Bone Scintigraphy Report

Clinical History

41-year-old female patient with history of breast cancer.
Complaining of generalized bone aches.

Technique

Following the intravenous administration of the radiopharmaceutical, static anterior and posterior whole-body skeletal images were acquired.

Findings

- **Skeletal Uptake**: There is diffusely, intensely increased radiopharmaceutical accumulation throughout the entire axial and proximal appendicular skeleton.
- **Soft Tissue & Renal Excretion**: There is a striking, near-complete absence of normal soft tissue background activity. Crucially, faint-to-absent physiological radiopharmaceutical excretion is noted in both kidneys and the urinary bladder.
- **Appendicular Skeleton**: Intense symmetric uptake extends into the humeri and femora, fading in the distal extremities.

Conclusion

Features are typical of a metastatic "superscan" appearance, secondary to diffuse, widespread osteoblastic skeletal metastasis from the known primary breast malignancy.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

NILE DIAGNOSTIC CENTER

Bone Scan
[[DOCTYPE:BONE_SCAN]]

5594

GUSTAVE ROUSSY INTERNATIONAL EGYPTE

Name : AML MOHAMED ABDELSALAM	**Date : 12-01-2026 12:45**
Branch : Portable	**Medical No. : GRI-28511180102922**
Ref.Physician Prof :	**Ins.Company :**

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

I- CHEST:

- Large right breast mass lesion measuring 78 x 33 mm with associated large right axillary lymph nodes, the largest measures 32 x 21 mm
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No dilated biliary radicles. Small right hepatic lobe segment VI focal lesion measuring 7 mm showed marginal enhancement
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Multiple lytic and sclerotic osseous lesions seen in bilateral femoral , iliac bones as well as dorsal and lumbar vertebral bodies
- **Opinion:**
  - **Right hepatic lobe small focal lesion ?? metastatic.**
  - **Multiple osseous metastatic lesions.**

Dr. Asser Ahmed

Dr. Asser Ahmed

CT Scan
[[DOCTYPE:CT_SCAN]]